BASELINE → TREATMENT → FOLLOW-UP

My treatment, tests and checks

Source-required blood tests are distinguished from tests ordered for a particular symptom or disease plan. Imaging is not interchangeable with blood safety tests.

Check	Before starting	Treatment visit	Between visits	Phase / dose review	After treatment
Blood and organ tests	Blood count, kidney and liver function; audiology and measured creatinine clearance when indicated. Review hearing, tinnitus, nerve symptoms and medicines. Dental and nutrition preparation and a swallowing plan are important before head and neck radiotherapy.	Blood count, kidney and liver function before every cisplatin cycle. Report hearing or tinnitus and nerve changes before each dose; audiology is repeated when indicated. Review weight, drinking, swallowing, mouth and skin symptoms during the radiation course.	Contact the team if symptoms develop; extra tests may be needed before the next visit.	Recheck relevant results after a hold or changed plan; previous toxicity remains relevant.	Follow kidney, mineral, hearing and nerve problems and continuing radiation effects. Dental, nutrition and swallowing care may remain necessary after the last dose. Scans, examinations and any thyroid or other late-effect tests are individualised by the radiation and oncology teams; no universal scan or blood tumour-marker schedule is specified here.
Radiotherapy	Planning appointment and separate radiation calendar.	Cisplatin before same-day radiation; hold cisplatin if radiation is cancelled.	Attend radiation on its own calendar; no assumption of weekend sessions.	Coordinate interruptions and recovery between both teams.	Late effects and response follow-up belong to the radiation plan.
Treatment and appointments	Confirm the exact diagnosis, intent, prescription and calendar.	Cisplatin is given by intravenous (IV) infusion with fluids before and after, prescribed mineral replacement and anti-sickness medicines. The unit confirms infusion and hydration times and checks that radiotherapy is proceeding that day. Allow time for both departments.	Use the prescribed support plan and contact route.	Mouth, skin, swallowing, energy and hydration may need support after radiation ends. The team arranges response assessment and follows hearing, kidney or nerve problems. Any further chemotherapy is a separate prescription.	Record the final dose and follow-up appointments.
Eating, mouth and swallowing	Agree oral care and assess weight, intake and swallowing; arrange dental review before head/neck radiation.	Review pain, weight, hydration and safe swallowing.	Report poor intake, choking, weight loss or inability to drink promptly.	Dietitian and speech/language plans adapt to symptoms and recovery.	Continue dental, nutritional and swallowing follow-up as needed.
Imaging and tumour assessment	Confirm staging and any relevant tumour tests; these are distinct from blood safety tests.	Discuss symptoms and treatment response.	The team arranges scans or examinations when indicated.	Use the agreed assessment to guide the next phase.	Record scan and clinic dates; this regimen does not impose a single timetable for everyone.

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. National Cancer Control Programme (NCCP) 00385 v5: official regimen

Tests in your plan

Full blood count, kidneys and liver

WHEN

Before every cisplatin cycle

Support safe treatment and identify problems needing review.

Hearing assessment

If part of your plan

WHEN

Audiology when indicated; report hearing or tinnitus changes before dosing.

Checks a medicine-specific risk that symptoms alone may miss.

THE WHOLE TREATMENT JOURNEY

Weekly cisplatin + head and neck radiotherapy: treatment and checks

Locally advanced unresectable squamous head and neck cancer when three-weekly cisplatin cannot be tolerated.

Cisplatin 40 mg/m^2 once a week for six planned doses during radiotherapy. Cisplatin is given before radiation on the same day. If radiotherapy is cancelled, cisplatin is held until radiation resumes; the team updates both calendars.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	Treatment visit	Between visits	Phase / dose review	After treatment
Blood and organ tests NCCP	Blood count, kidney and liver function; audiology and measured creatinine clearance when indicated. Review hearing, tinnitus, nerve symptoms and medicines. Dental and nutrition preparation and a swallowing plan are important before head and neck radiotherapy.	Blood count, kidney and liver function before every cisplatin cycle. Report hearing or tinnitus and nerve changes before each dose; audiology is repeated when indicated. Review weight, drinking, swallowing, mouth and skin symptoms during the radiation course.	Contact the team if symptoms develop; extra tests may be needed before the next visit.	Recheck relevant results after a hold or changed plan; previous toxicity remains relevant.	Follow kidney, mineral, hearing and nerve problems and continuing radiation effects. Dental, nutrition and swallowing care may remain necessary after the last dose. Scans, examinations and any thyroid or other late-effect tests are individualised by the radiation and oncology teams; no universal scan or blood tumour-marker schedule is specified here.
Radiotherapy NCCP	Planning appointment and separate radiation calendar.	Cisplatin before same-day radiation; hold cisplatin if radiation is cancelled.	Attend radiation on its own calendar; no assumption of weekend sessions.	Coordinate interruptions and recovery between both teams.	Late effects and response follow-up belong to the radiation plan.
Treatment and appointments NCCP	Confirm the exact diagnosis, intent, prescription and calendar.	Cisplatin is given by IV infusion with fluids before and after, prescribed mineral replacement and anti-sickness medicines. The unit confirms infusion and hydration times and checks that radiotherapy is proceeding that day. Allow time for both departments.	Use the prescribed support plan and contact route.	Mouth, skin, swallowing, energy and hydration may need support after radiation ends. The team arranges response assessment and follows hearing, kidney or nerve problems. Any further chemotherapy is a separate prescription.	Record the final dose and follow-up appointments.
Eating, mouth and swallowing NCCP	Agree oral care and assess weight, intake and swallowing; arrange dental review before head/neck radiation.	Review pain, weight, hydration and safe swallowing.	Report poor intake, choking, weight loss or inability to drink promptly.	Dietitian and speech/language plans adapt to symptoms and recovery.	Continue dental, nutritional and swallowing follow-up as needed.

Care / check	Before starting	Treatment visit	Between visits	Phase / dose review	After treatment
Imaging and tumour assessment NCCP	Confirm staging and any relevant tumour tests; these are distinct from blood safety tests.	Discuss symptoms and treatment response.	The team arranges scans or examinations when indicated.	Use the agreed assessment to guide the next phase.	Record scan and clinic dates; this regimen does not impose a single timetable for everyone.

What each check is for

1 Cisplatin with radiotherapy

6 weekly doses

Weekly cisplatin supports radiotherapy for locally advanced head and neck cancer when the three-weekly cisplatin schedule cannot be tolerated. The team explains why this approach fits your situation.

Ask / act: Confirm the current phase, prescription, tests and support plan.

Sources: NCCP

2 Recovery and follow-up

After concurrent treatment

Weekly cisplatin supports radiotherapy for locally advanced head and neck cancer when the three-weekly cisplatin schedule cannot be tolerated. The team explains why this approach fits your situation.

Ask / act: Confirm the current phase, prescription, tests and support plan.

Sources: NCCP

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Sources checked 2026-11-07 · Source reconciled draft; independent clinical, pharmacy and patient/PPI review pending · SACTCheck v0.84.45